



TARGET POPULATION: Adults ≥ 18 years

RISK FACTORS

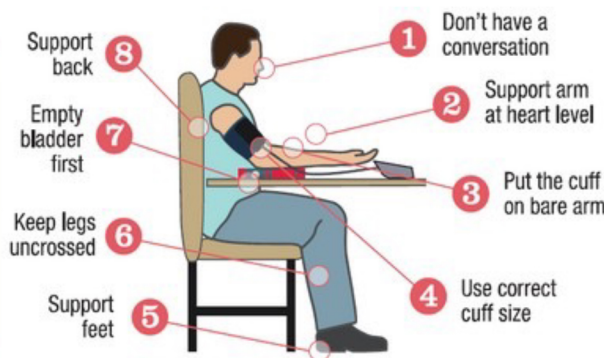
- Age > 40 years of age
- Present profile: Diabetes Mellitus and/or Chronic Kidney Disease(Refer to DM & CKD leaflet), High Waist Circumference (≥ 80 cm in women, ≥ 90 cm in men), BMI(kg/m²)>23, Sedentary lifestyle (no or less physical activity)
- Past history: Gestational Hypertension
- Personal history: Smoking and Alcohol Consumption
- Family history: Ischemic Heart Disease or Stroke

DIAGNOSIS

ACCURATE BLOOD PRESSURE MEASUREMENT

Ask to sit quietly for *five minutes* with back supported by the chair.

The bladder of the cuff should encircle *two-thirds* of the arm circumference.



Ref: Campbell et. al. 2022

ACCURATE BLOOD PRESSURE
MEASUREMENT VIDEO



01 ESSENTIAL DRUG LIST AND DOSE WORLD HEALTH ORGANIZATION

<i>Amlodipine*</i>	5-10 mg
<i>Enalapril /Losartan*</i>	5-10 mg/25-50 mg
<i>Hydrochlorothiazide*</i>	12.5 mg

Use Chlorthalidone 6.25-12.5 mg instead of Hydrochlorothiazide 6.25-12.5 mg if available

**National list of essential medicines, 6th revision, MOHP Nepal*

02 COMMON SIDE EFFECTS

<i>Amlodipine</i>	Ankle swelling
<i>Enalapril /Losartan</i>	Dry cough
<i>Hydrochlorothiazide</i>	Low sodium

03 CONTRAINDICATIONS

In cases of **Pregnancy**, **Kidney Failure** and **High Potassium**, drug Enalapril/Losartan should not be used.

Never prescribe Enalapril and Losartan in combination

Assess for adherence to treatment at each visit

High blood pressure in young adults could be due to kidney disease

ALSO REFER TO
KHDC CVD RISK ASSESSMENT

OUR COLLABORATORS



Ref. PEN Manual (MoHP), 2075



Hôpitaux
Universitaires
Genève

Disclaimer: This leaflet is currently being piloted and may contain inconsistencies. Please help us improve it by reporting any issues to KHDC Nepal.



**KHDC
NEPAL**



HYPERTENSION IDENTIFICATION AND MANAGEMENT FOR PRIMARY CARE PROVIDERS

USE THIS GUIDE TO

- Identify patients with Hypertension
- Screen for Hypertension among patient at risk
- Manage and refer patients with Hypertension

Hypertension is defined as Systolic Blood Pressure (SBP) of ≥ 130 mmHg and/or Diastolic blood pressure (DBP) of ≥ 80 mmHg on at least 2 visits at least 1 week apart.

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INDICATIONS FOR TREATMENT

BLOOD PRESSURE	CVD RISK <20%	CVD RISK 20-30%	CVD RISK >30%
SBP: 130-139 mmHg and/or DBP: 80-89 mmHg	F/U 3 Monthly		
SBP: 140-159 mmHg and/or DBP: 90-99 mmHg	Start Amlodipine 5mg OD Atorvastatin 10-20mg		Start Amlodipine 5mg OD and Add Atorvastatin 10-20 mg
SBP ≥ 160 mmHg and/or DBP ≥ 100 mmHg		Start Amlodipine 10 mg OD and Add Atorvastatin 10-20 mg	

Refer to CVD risk assesment leaflet



TARGET BLOOD PRESSURE

General population <140/90

Diabetes mellitus, chronic kidney disease <130/80

Note: less stringent BP control for frailty

Ref: European Society of Cardiology and European Society of Hypertension (2018/2023)



INVESTIGATIONS

If investigations are not available, it should not delay initiation of treatment.

Urinary protein | Serum creatinine
ECG | Lipid Profile (fasting)
FBS | HbA1c (if available)

Always test serum creatinine and **potassium*** in patients prescribed with Enalapril/Losartan in 2 weeks ***if available** if higher potassium or creatinine. - Refer to higher center



NON-PHARMACOLOGICAL ADVICE



PHARMACOLOGICAL TREATMENT

Tab. Amlodipine 5- 10 mg once a day

1 WEEK



Patient above target blood pressure with adherence to treatment

Add Tab. Enalapril 5-10 mg or Losartan 25-100 mg

1 MONTH



Patient above target blood pressure with adherence to treatment

Add Tab. of Hydrochlorothiazide 12.5 mg once a day

1 MONTH



Patient above target blood pressure with adherence to treatment - REFER

FOLLOW-UP INTERVALS

1st follow-up after 1 week

Monthly follow-up

Until patient reaches target blood pressure

3-6 month follow-up

Once target blood pressure is reached

REFERRAL CRITERIA

- If SBP>180mmHg and/or DBP>110mmHg (Give amlodipine 5 mg PO stat & refer)
- Features of end organ damage include: Chest pain (angina); difficulty breathing, blurring of vision, and chronic kidney disease.
- Uncontrolled blood pressure: Not controlled with 3 drugs: amlodipine and enalapril/ losartan and hydrochlorothiazide after inquiring about compliance.
- Age <30 years with no family history of hypertension.
- Associated with unexplained hypokalemia, severe hyponatremia or hyperkalemia.
- Pregnancy with Hypertension
- Assymetrical BP in 2 arms with > 20-30 mm difference

“ HYPERTENSION IS A SILENT KILLER ”

CONTACT US

For any clinical queries / If anything in the document is difficult to understand



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